A GUIDE FOR PATIENTS

Releasing Pelvic Pain

Understanding chronic pelvic pain and pelvic floor tension, and taking an active part in your recovery

Dr. Nicholas Power
Urologist, London Health Sciences Centre
Victoria Hospital, London, Ontario
Office: 1-888-909-2786

For education only. This guide does not replace advice from your own care team. October 2026. Read online at nicholaspowerurology.com/education/releasing-pelvic-pain

Contents

BEFORE YOU BEGIN	You are not imagining this	3
CHAPTER 1	What is chronic pelvic pain syndrome?	5
CHAPTER 2	Why your tests keep coming back normal	8
CHAPTER 3	Meet your pelvic floor	10
CHAPTER 4	The cycle that keeps pain going	13
CHAPTER 5	A sensitive alarm system	15
CHAPTER 6	Stories from the clinic	17
CHAPTER 7	You have more control than you think	22
CHAPTER 8	Pelvic floor physiotherapy	24
CHAPTER 9	Learning to let go: your daily practice	26
CHAPTER 10	Calming the alarm system	31
CHAPTER 11	Everyday habits that make a difference	33
CHAPTER 12	Medications and other treatments	36
CHAPTER 13	Handling flare-ups	38
CHAPTER 14	When to get help sooner	39
CHAPTER 15	Working with your care team, and further resources	40
A FINAL WORD	Recovery is possible	41
WORKSHEETS	Your recovery tools	42

BEFORE YOU BEGIN

You are not imagining this

If you are reading this guide, you have probably been living with pain or discomfort somewhere between your belly button and your thighs for a long time. It might be an ache deep behind the scrotum or in the vagina, a burning at the tip of the penis, pressure over the bladder, a feeling of sitting on a golf ball, or a pull into the testicles, groin, tailbone or rectum. It might come with a constant urge to pee, a weak or stop-start stream, pain after sex, or simply a feeling that something "down there" is not right.

You have probably also had tests. Urine tests, swabs, blood work, an ultrasound, maybe a scope or an MRI. Many people with this kind of pain have been given several courses of antibiotics. And very often the answer that comes back is some version of *"Everything looks normal."*

That answer can feel like a door closing. If everything is normal, why does it hurt so much? Is something being missed? Do people think you are making it up?

Here is what I want you to know before you read another word: **your pain is real, it is common, and it is treatable.** Normal test results do not mean nothing is wrong. They mean the problem is very unlikely to be an infection, cancer or damage that a scan can see. In most people, the problem is in how the muscles of the pelvis and the nervous system that controls them are behaving. Muscles and nerves can change. That is why this condition can get better, and why **you have more influence over your recovery than you probably realize.**

I wrote this guide because I see many patients in my clinic who have been through months or years of tests, treatments and worry before anyone sat down and explained what is going on. Again and again, I see the same thing: once people understand their pain, they become less afraid of it, and once they are less afraid of it, they start to get better. Understanding is not a side dish here. It is part of the treatment.

This guide will walk you through:

- what chronic pelvic pain syndrome is, and why your tests come back normal;
- the role of the pelvic floor muscles, and how they can get "stuck" in a tight, protective state;
- the cycle of tension, worry and pain that keeps symptoms going;
- stories of patients who have recovered, and what helped them;
- the daily practices that make the biggest difference;
- what to expect from physiotherapy, medication and other treatments;
- how to handle flare-ups without losing hope;
- and the warning signs that mean you should be seen sooner.

Read it at your own pace. You do not need to do everything at once. Some people read it in one sitting; others read a chapter a week. Many find it helpful to read it twice: once to understand, and again a few weeks later to put it into practice. If you have a partner or family member who wants to understand what you are going through, it may help them to read it too.

Dr. Nicholas Power
Urologist, London Health Sciences Centre, London, Ontario

About this guide. This guide is for education. It does not replace an assessment by your own doctor, and it is meant for people who have already been checked for other causes of their symptoms. The patient stories are composites: they are drawn from common experiences, but the names and details are invented, and none of them describes a real person. If you have any of the warning signs in “When to get help sooner”, contact your doctor or go to an emergency department.

CHAPTER 1

What is chronic pelvic pain syndrome?

Chronic pelvic pain syndrome means pain or discomfort in the pelvic area that has lasted at least three months, where careful testing has not found an infection, a stone, a tumour or another clear cause. “Chronic” simply means long-lasting. It does not mean permanent, and it does not mean severe. “Syndrome” means a recognizable pattern of symptoms that tend to travel together.

Over the years, this condition has been given many names, and you may have heard some of them:

- **Chronic prostatitis** or **chronic prostatitis/chronic pelvic pain syndrome (CP/CPPS)**, in men. Despite the name, the prostate is usually not infected and is often not the source of the pain at all.
- **Interstitial cystitis** or **bladder pain syndrome**, when the pain is felt mainly in the bladder and comes with frequent, urgent urination. This is more common in women but also happens in men.
- **Pelvic floor tension**, **pelvic floor myalgia**, **levator ani syndrome** or **hypertonic pelvic floor**, which all describe pelvic floor muscles that are too tight and tender.
- Other related names include **chronic scrotal pain**, **chronic urethral pain** and **proctalgia** (rectal pain).

These labels overlap a great deal. Two people with the same symptoms can be given different names depending on which doctor they see. What matters more than the label is understanding what is driving your pain, because that is what guides treatment.

You are far from alone

Chronic pelvic pain is one of the most common reasons people see a urologist. Studies suggest that about 1 in 12 men will have symptoms of chronic prostatitis/chronic pelvic pain syndrome at some point, and bladder pain syndrome affects millions of women across North America. It can begin at any adult age, but it often starts in the 20s to 50s, at a time when people are busy with work, relationships and families. Because pelvic pain is private and hard to talk about, many people suffer quietly and assume they are the only one. You are not.

Where the pain shows up

Pelvic pain rarely stays in one neat spot. It often moves around, changes from day to day, and can be felt in places that seem unrelated. Common places include:

- the **perineum**, the area between the genitals and the anus (people often describe this as “sitting on a ball” or a deep ache “inside”);
- above the pubic bone, over the **bladder**;

- the **penis**, especially the tip, or the **testicles** and scrotum;
- the **vagina**, vulva or opening of the urethra;
- the **rectum** or **tailbone**;
- the **groin**, inner thighs, hips or low back.

Other symptoms that often come along

- **Bladder symptoms:** needing to pee often, urgency, getting up at night, a weak or hesitant stream, a feeling of not emptying, or burning with urination.
- **Sexual symptoms:** pain during or after ejaculation or orgasm, pain with intercourse, changes in erections or arousal, or losing interest in sex because of fear of pain.
- **Bowel symptoms:** constipation, straining, pain with bowel movements, or a feeling of pressure in the rectum.
- **Sitting pain:** discomfort that builds the longer you sit, and eases when you stand or lie down.
- **Mood and sleep:** worry, low mood, irritability, poor sleep and feeling worn down. These are a very normal response to living with pain, and as you will see, they also feed back into the pain itself.

The pattern is often the clue

Many people notice that their pain:

- is milder in the morning and builds through the day;
- gets worse with long periods of sitting, driving or cycling;
- flares during stressful weeks, after poor sleep, or after an argument;
- can flare a day or two after sex, a hard workout, constipation or a long car trip;
- eases with warmth, a hot bath, a walk, a holiday, or when they are absorbed in something enjoyable.

This pattern is very typical of *muscle* pain, and of pain that is strongly affected by the nervous system. It is not the pattern of an infection or a tumour, which do not take holidays.

Key points

- Chronic pelvic pain syndrome is pain lasting three months or more without an infection, tumour or other clear cause.
- It has many names, but they describe overlapping versions of the same problem.
- It is very common, in both men and women.
- Pain that moves around, builds with sitting, and flares with stress is typical, and points toward the muscles and nerves.

CHAPTER 2

Why your tests keep coming back normal

When something hurts, it is natural to assume that something must be damaged, infected or diseased, and that the right test will find it. With chronic pelvic pain, that search can go on for years. Each normal result brings a moment of relief, followed by frustration: *Then why do I still hurt?*

Here is the key idea: **the tests we use are designed to find infection, inflammation, blockage and cancer. They are not designed to measure muscle tension or how sensitive your nerves have become.** A urine culture can find bacteria. An ultrasound can see a cyst or a mass. A scope can see the inside of the bladder. None of them can show a pelvic floor muscle that has been clenched for months, or a nervous system that has turned its alarm volume up. Those problems are real, but they are invisible on standard tests.

So when your tests are normal, that is genuinely good news. It tells us what you almost certainly *don't* have:

- You do not have an ongoing infection that needs more antibiotics.
- Cancer is very unlikely to be the cause of this pain.
- You do not have a blockage or structural damage that needs an operation.

And it points us toward what you very likely *do* have: a problem of muscle tension and nerve sensitivity, which responds to a different kind of treatment.

What about the antibiotics?

Many people with pelvic pain have been treated with antibiotics, sometimes many times, often for a diagnosis of "prostatitis" or "bladder infection." Sometimes the symptoms improve briefly, which makes it seem as if the antibiotics worked. That short improvement is often explained by other things: time passing, less worry once treatment starts, the anti-inflammatory effect some antibiotics have, or the natural ups and downs of pelvic pain.

In true chronic pelvic pain syndrome, the cultures are negative because there is no infection. One reasonable trial of antibiotics early on is sometimes appropriate. Repeating them over and over is not: it rarely helps, it exposes you to side effects, it can upset your bowels (which often makes pelvic pain worse), and it keeps you searching for an infection that isn't there. If your cultures are negative, it is usually time to stop looking for a germ and start treating the muscles and nerves.

"Is something being missed?"

This is the question I hear most often, and it is a fair one. Before making this diagnosis, your doctor will have looked for the conditions that can mimic pelvic pain. Depending on your symptoms, that

may include urine tests and cultures, tests for sexually transmitted infections, a physical exam, blood work such as a PSA in men, an ultrasound, a scope of the bladder, or a scan.

Once those have been done and are reassuring, **more testing usually adds worry rather than answers.** Each new test is another chance for an incidental finding (a harmless small cyst, for example) that leads to more anxiety and more tests. If your symptoms change in an important way, or you develop one of the warning signs listed in “When to get help sooner”, you should be checked again. Otherwise, it is usually best to put your energy into treatment rather than into the search.

Key points

- Normal tests make infection, cancer and structural damage very unlikely. That is good news.
- Standard tests cannot measure muscle tension or nerve sensitivity, which are the usual drivers of this pain.
- Repeated antibiotics rarely help when cultures are negative.
- Once the right tests have been done, the focus should shift from searching to treating.

CHAPTER 3

Meet your pelvic floor

To understand pelvic pain, it helps to understand the muscles at its centre.

Your **pelvic floor** is a group of muscles that stretches across the bottom of your pelvis, from the pubic bone at the front to the tailbone at the back, and from one sitting bone to the other. Think of it as a hammock or a trampoline that holds up your bladder, bowel and, in women, the uterus. Openings pass through it: the urethra (where urine comes out), the anus, and in women the vagina.

These muscles do a lot of work you never think about:

- They **hold things in**, by squeezing around the urethra and anus so you don't leak urine, stool or gas.
- They **let things out**, by relaxing completely when you pee, have a bowel movement or, in women, during childbirth and comfortable intercourse.
- They **support your organs** and help stabilize your spine and hips, working as a team with your deep belly muscles, your diaphragm (the breathing muscle under your ribs) and your back muscles.
- They play a key role in **sexual function**, including erection, ejaculation, arousal and orgasm.

Healthy pelvic floor muscles are like healthy hands: they can grip firmly when needed and then open fully and rest. Problems arise when they can do only one of those things.

Too weak, or too tight

Most people have heard of the pelvic floor in connection with *weakness*: bladder leakage after childbirth, or after prostate surgery. Weak muscles are treated with strengthening exercises, often called Kegels.

In chronic pelvic pain, the problem is usually the opposite. The muscles are **too tight, too often**. They are holding on when they should be resting. They may be tender to touch, contain tight, sore knots (called **trigger points**), and have trouble letting go fully even when you try. A physiotherapist or doctor examining these muscles often finds them firm and tender, and pressing on a trigger point can reproduce exactly the pain you know, sometimes felt in a different place, such as the tip of the penis, the testicle, the bladder or the vagina. This is called **referred pain**, and it explains why pain can be felt in organs that are perfectly healthy.

A note about Kegel exercises. If your pelvic floor is tight and painful, standard Kegel (squeezing) exercises can make your pain worse, because they train the muscles to tighten even more. Many people with pelvic pain have tried Kegels after reading about them online and found they made things worse. Unless a pelvic floor physiotherapist has assessed you and recommended them, **don't do strengthening exercises for pelvic pain.** Your first job is to learn to let these muscles go.

A clenched fist you can't see

Try this. Make a fist and squeeze it as tightly as you can. Hold it. After thirty seconds it starts to ache. After a minute it hurts. Now imagine holding it, not at full strength but at a steady half-squeeze, all day and through the night, for months. Your hand would ache, burn and feel tender. It would be stiff and hard to open. It would hurt more when you used it, and it would hurt more when you were tired or stressed.

Now imagine that the fist is hidden inside your body, where you cannot see it, cannot easily feel that it is clenched, and where it is wrapped around your urethra, your rectum and the nerves to your genitals. That is a fair picture of what many people with chronic pelvic pain are living with.

A tension headache is a good comparison. Most people understand that tight muscles in the neck, scalp and jaw can cause a pounding, real, sometimes disabling headache, and that a brain scan will be normal. The pelvis can do the same thing. The muscles are different, but the process is very similar.

This comparison, and the idea of a cycle of tension, anxiety and pain described in the next chapter, were made popular by Dr. David Wise and Dr. Rodney Anderson in their book *A Headache in the Pelvis*, which I recommend to many of my patients (see Chapter 15).

How the pelvis learns to clench

The pelvic floor is one of the places the body tightens when it feels threatened. This is an ancient protective reflex. Animals tuck their tails and tighten their pelvis when they are afraid. Humans do the same thing, usually without noticing: we clench the jaw, raise the shoulders, hold the breath, tighten the belly and squeeze the pelvic floor when we are stressed, rushed, anxious, cold, in pain, or concentrating hard.

For most people, this tightening comes and goes. But some people, for reasons of temperament, habit, life circumstances or a triggering event, start to hold this tension most of the time. Over months and years, the muscles adapt to being held short and tight. They become tired, tender and full of trigger points, and the nervous system that controls them gets into the habit of keeping them "on."

Common things that start or feed pelvic floor tension include:

- a period of high stress, worry, grief or overwork;
- a painful event in the pelvis, such as a urinary infection, a sexually transmitted infection, a kidney stone, a procedure, a hemorrhoid, a fissure, childbirth or surgery, which teaches the pelvis to “guard”;
- long hours of sitting, especially on hard chairs, in a car, or on a bicycle seat;
- constipation and straining;
- “holding it” for long periods, or going to the bathroom far too often “just in case”;
- heavy lifting, intense core training or strengthening exercises done with a braced, breath-holding style;
- painful sex, or worry about sex;
- a lifelong tendency to hold tension in the body, which many people with pelvic pain recognize in themselves once it is pointed out.

Often there is no single cause. Several of these come together, and one day the pelvis tips from “tight sometimes” to “tight most of the time.”

Key points

- The pelvic floor is a hammock of muscles that holds things in, lets things out, and supports sexual function.
- In chronic pelvic pain, these muscles are usually too tight and tender, not too weak.
- Tight pelvic floor muscles can refer pain to the bladder, penis, testicles, vagina, rectum and groin.
- The pelvis tightens as a protective reflex during stress and pain. In some people this becomes a constant habit.
- Avoid Kegel strengthening exercises for pelvic pain unless a physiotherapist recommends them.

CHAPTER 4

The cycle that keeps pain going

If tight muscles were the whole story, the pain would be simple to fix: just relax them. The reason chronic pelvic pain lingers is that it becomes a **cycle**, where each part feeds the next.

1. **Tension.** The pelvic floor tightens: from stress, from a painful event, from sitting, from habit.
2. **Pain.** Tight, tired muscles and irritated nerves hurt, often in confusing places.
3. **Worry.** Pain in the genitals or bladder is frightening. Is it cancer? An infection? Will it ever stop? What does it mean for my sex life?
4. **Protection.** Worry and pain trigger the body's threat response. Muscles brace even harder, breathing gets shallow, sleep suffers, and attention locks onto the pelvis.

...and more protection means more tension, which brings the cycle back to the start.

Each turn of the cycle makes the next turn easier. After a while, the cycle can run on its own, even after the event that started it is long gone. The infection has cleared, the stressful job has ended, the bike has been sold, but the pelvis is still clenched and the alarm is still ringing.

Worry is not the cause, but it is fuel

Please hear this carefully: **saying that stress and worry are part of the cycle does not mean the pain is "all in your head."** The pain is in your pelvis. The muscles are physically tight. The nerves are physically irritated. But the nervous system that controls those muscles is the same nervous system that feels fear. When you worry about your pain, your body responds as if it is under threat, and one of the things it does under threat is tighten the pelvis.

This is actually hopeful. It means that working on the worry part of the cycle, through understanding, reassurance, relaxation and sometimes counselling, has a direct, physical effect on the muscles. It gives you a handle you can grab.

Habits that keep the cycle turning

When we are in pain, we naturally do things to protect ourselves. Many of these make sense in the short term but keep the cycle going in the long term:

- **Checking:** repeatedly examining the testicles or genitals, testing whether it still hurts when you press, or searching the internet for causes. Each check focuses the nervous system on the pelvis and keeps the alarm on.
- **Bracing:** sitting carefully, holding the pelvis still, clenching during bowel movements or before sex, in anticipation of pain.
- **Avoiding:** giving up exercise, sex, social events or work out of fear that they will make the pain worse. Life gets smaller, mood drops, and pain takes up more room.
- **Over-doing on good days:** catching up on everything when you feel well, then flaring and feeling defeated.
- **Going to the bathroom "just in case":** which can train the bladder to hold less and complain sooner.

Recognizing these patterns is the first step to changing them. You don't have to change them all at once, and you don't have to be perfect.

Breaking the cycle

The good news about a cycle is that you can break it at more than one point. Treatment works on every part of it at the same time:

- **Tension** is treated with pelvic floor physiotherapy, daily relaxation practice, stretching and changes to sitting and bathroom habits.
- **Pain** is eased with heat, sometimes medication, and over time by the muscles recovering.
- **Worry** is reduced by understanding what is happening (which is why this guide exists), by a clear plan, and sometimes by counselling or pain psychology.
- **Protection** is calmed by slow breathing, better sleep, gentle activity and gradually returning to the things you have been avoiding.

Key points

- Chronic pelvic pain is kept going by a cycle of tension, pain, worry and protective bracing.
- The cycle can continue long after whatever started it has gone.
- Stress does not mean the pain is imaginary; it means the nervous system is involved, which gives you a way to influence it.
- Treatment works on every part of the cycle at once.

CHAPTER 5

A sensitive alarm system

There is one more piece to the puzzle, and it explains why pelvic pain can feel so much bigger than the muscle tension behind it.

Pain is produced by your nervous system as a **warning**. Its job is to protect you, not to give you an exact report of damage. Most of the time the two match: you burn your hand, and it hurts. But pain and damage are not the same thing. A paper cut can hurt more than a deep wound noticed later. A soldier can be injured in battle and feel nothing until later. And someone with long-lasting pain can feel severe pain with no new damage at all.

When pain goes on for months, the nervous system **learns**. It becomes better and faster at producing pain. Nerves in the pelvis become more easily triggered, the spinal cord amplifies their signals, and the brain pays closer attention to anything coming from the pelvis. Doctors call this **central sensitization**. A simpler way to picture it is a **smoke alarm that has become too sensitive**: instead of going off only when there is a fire, it goes off when you make toast.

A sensitized alarm system explains many of the confusing features of pelvic pain:

- why a normal amount of urine in the bladder can feel urgent or painful;
- why light touch, tight clothing or sitting can be uncomfortable;
- why ejaculation, which should feel good, can be followed by hours of aching;
- why the pain can spread, move around, or show up in places with nothing wrong with them;
- why stress, poor sleep and worry make pain worse: they turn up the alarm's sensitivity;
- why people with pelvic pain often also have irritable bowel syndrome, tension headaches, jaw pain, back pain or fibromyalgia, which involve the same kind of sensitivity.

Hurt does not mean harm

This may be the single most important idea in this guide: **in chronic pelvic pain, more pain does not mean more damage.** A flare after sitting, sex or a hard week does not mean you have injured yourself or that something new has gone wrong. It means the alarm is sensitive and the muscles are irritable. It will settle.

When people truly understand this, something changes. The fear that comes with each flare gets smaller. They stop checking, stop bracing, and start returning to normal life. And as the threat response quiets down, the alarm gradually becomes less sensitive. Learning about pain is not just reassuring; research in many kinds of chronic pain shows that it reduces pain.

The alarm can be turned down

Nervous systems that can learn to be more sensitive can also learn to be less sensitive. The same things that help the pelvic floor also help the alarm system settle:

- understanding that pain is not damage;
- regular, gentle relaxation of the body and the breath;
- restful sleep;
- regular, comfortable movement and exercise;
- doing things you enjoy and that absorb your attention;
- social connection and support;
- in some cases, medications that calm overactive nerves.

This is slow work. The nervous system changes over weeks and months, not days. But it does change.

Key points

- Long-lasting pain makes the nervous system more sensitive, like a smoke alarm that goes off for toast.
- This sensitivity explains urgency, pain after ejaculation, sitting pain and pain that moves around.
- In chronic pelvic pain, more pain does not mean more damage.
- The nervous system can be retrained to be less sensitive, over weeks to months.

CHAPTER 6

Stories from the clinic

The stories below are composites. Each one is drawn from patterns I see often, but the names and details are invented. You may find that one of them sounds a lot like you. Notice what helped each person: it is almost never one thing, and it is almost never quick.

STORY 1

Mark: the antibiotics that never quite worked

Mark was 38, a software developer and a father of two young children. His trouble began during a difficult stretch at work, with long days at his desk and many late nights. He noticed a deep ache behind his scrotum, “like sitting on a tennis ball,” that was worse by the end of the day. Soon he was getting up twice a night to pee, and his stream seemed weaker. Ejaculation was followed by a burning ache that lasted into the next day.

His family doctor diagnosed prostatitis and prescribed four weeks of antibiotics. He felt a little better for a while, then worse. Over the next year he had three more courses. Each urine culture came back negative. He searched online late at night and became convinced he had a hidden infection, or something worse. He stopped going to the gym, avoided sex, and had trouble concentrating at work.

When I examined Mark, his prostate was normal in size and not inflamed, but the muscles around it were tight and very tender. Pressing on one spot reproduced the exact ache he felt when he sat. His PSA, ultrasound and scope were all normal.

The turning point for Mark was understanding the problem. “So it’s like a cramp,” he said, “and I’ve been sitting on it all day.” He stopped antibiotics. He started seeing a pelvic floor physiotherapist every two weeks, and practised a relaxation routine at home for twenty minutes every evening. He set a timer to stand up every 30 minutes at work and bought a cushion with a cut-out. He took a medication to relax the bladder outlet for a few months, which helped his stream and his night-time trips. He went back to walking, then to the gym, but switched from heavy lifting to swimming for a while.

Progress was uneven. At four months he had a bad flare during a deadline week and was sure he was back to square one. His physiotherapist helped him see that the flare was shorter and milder than his old ones. By nine months, most days were good days. He still notices a twinge after a long drive, and he knows what to do about it.

What helped: understanding the diagnosis, stopping the search for an infection, physiotherapy, daily relaxation, changing how he sat, and patience through a flare.

STORY 2

Diane: the bladder that wouldn't settle

Diane was 54 and worked in a busy retail store. For two years she had felt pressure and burning over her bladder, and had been going to the bathroom every 45 minutes, day and night. She planned every outing around washrooms. She had been treated for "bladder infections" many times, but most of her urine cultures were negative. Her scope showed a normal bladder lining, with no ulcers.

Diane was diagnosed with bladder pain syndrome. On examination, the muscles of her pelvic floor were tight and tender, and pressing on them recreated the bladder pressure she felt every day.

Her plan had several parts. She kept a food and symptom diary for a few weeks and found that coffee, cola and orange juice reliably made her bladder worse; tea in the afternoon was fine. She drank water steadily through the day instead of avoiding fluids, which had been making her urine concentrated and more irritating. She saw a pelvic floor physiotherapist who used hands-on treatment to release the tight muscles, and she learned slow belly breathing to use when urgency hit. She started a low dose of a nerve-calming medication at bedtime, which helped her sleep through the night. Once the pain had eased, she gradually stretched the time between bathroom trips, a few minutes at a time.

After six months, Diane was going every two to three hours and sleeping most nights without getting up. She still has flares, usually during stressful weeks at work or after a night of poor sleep, but she has a written flare plan on her fridge and the flares rarely last more than a few days.

What helped: identifying trigger foods and drinks, drinking enough water, pelvic floor physiotherapy, breathing to calm urgency, a medication to help sleep and nerve pain, and slowly retraining the bladder.

STORY 3

Raj: the fear that made everything louder

Raj was 27, very fit, and lifted weights five days a week. After a heavy deadlift session he felt a sharp pain in his groin that spread into his left testicle and the tip of his penis. The sharp pain faded, but a dull ache stayed. He became terrified that it was testicular cancer. He

checked his testicles several times a day, saw three different doctors, and had two ultrasounds. Both were completely normal.

The reassurance never lasted. Every twinge sent him back to checking and searching. He had read online that Kegel exercises help the prostate, so he did hundreds of squeezes a day. The pain got worse, and now he had urgency and pain after ejaculation too.

When we talked, Raj recognized the cycle right away. His heavy, breath-holding lifting style, his hours of Kegels and his constant checking had all been tightening his pelvis. His fear was turning up the alarm. He agreed to stop the Kegels completely and to stop checking his testicles every day, with a plan to see his doctor promptly if he ever noticed a genuinely new lump. He worked with a physiotherapist on releasing the muscles and learning to breathe out during effort. He also saw a counsellor for a few sessions to work on the health anxiety that kept pulling him back to the internet.

Within three months his pain was mostly gone. He returned to lifting with lighter weights and better breathing, and he has stayed well.

What helped: stopping the Kegels and the checking, physiotherapy, learning to breathe during effort, and treating the health anxiety that was feeding the cycle.

STORY 4

Sarah: when the body learns to guard

Sarah was 31. After a year with several genuine, culture-proven bladder infections, the infections stopped, but the symptoms didn't. She felt burning at the urethra, a constant mild urge to pee, and pain with intercourse that made her dread being intimate with her husband. She had begun tightening up in anticipation of pain, which made sex more painful, which made her tighten more.

Her pelvic exam showed no infection and no skin problems, but her pelvic floor muscles were firm and tender, particularly near the opening of the vagina and around the urethra. Her body had learned to guard after a year of pain, and had not unlearned it.

Sarah worked with a pelvic floor physiotherapist on releasing the muscles, including gentle internal treatment and, later, a home program with graduated vaginal trainers. She and her husband agreed to take intercourse off the table for a while and focus on intimacy without pressure, which removed a major source of dread. She used a good lubricant when they resumed. She learned to recognize the moment she started to brace, and to breathe and let go instead. Her urinary symptoms improved first, at around eight weeks. Comfortable intercourse took longer, about six months, and came back gradually.

What helped: understanding that the pain was guarding, not ongoing infection; physiotherapy with a home program; removing pressure around sex; and good communication with her partner.

STORY 5

Bill: it started in the bowels

Bill was 66 and retired. He had a long history of constipation and had been straining on the toilet for years. Over several months, he developed a deep aching pain in his rectum and tailbone, with a feeling of pressure “like something is stuck,” and a slow, hesitant urine stream. It was worst after bowel movements and when sitting in his favourite recliner.

In a man of Bill’s age, a slow stream is most often caused by the prostate, so that was checked first. His colonoscopy, prostate exam, PSA, urine flow test and bladder scan were all reassuring, apart from tight, tender pelvic floor muscles and a bladder that did not quite empty because the muscles weren’t letting go.

For Bill, fixing his bowels came first. He increased fibre slowly, drank more water, and used a footstool on the toilet so that his knees were higher than his hips. He stopped straining and stopped spending long periods on the toilet. His family doctor added a gentle daily laxative (polyethylene glycol). A physiotherapist taught him how to relax and “bulge” the pelvic floor during bowel movements instead of pushing against tight muscles, and how to relax fully while peeing. He started walking every day, and he replaced the recliner with a firmer chair for part of the day.

Within three months, his bowel movements were soft and easy, his urine stream was stronger, and his rectal ache had faded to an occasional twinge.

What helped: soft, regular bowel movements without straining, a footstool, learning to relax the pelvic floor on the toilet, daily walking and changing how he sat.

What these stories have in common

- Each person had real pain, and normal or reassuring tests.
- Each had tight, tender pelvic floor muscles.
- Something started the problem (stress, an infection, heavy lifting, constipation), and the cycle of tension, pain and worry kept it going.
- Understanding the problem was the first turning point.
- Recovery came from several changes working together, practised consistently.

- Recovery took months, not weeks, and included setbacks.
- Each person ended up with a set of skills they can use if symptoms return.

CHAPTER 7

You have more control than you think

Chronic pelvic pain often makes people feel helpless. It seems to come and go on its own schedule, and treatments that work for someone else don't seem to work for you. But look again at the cycle in Chapter 4 and the stories in Chapter 6. Almost every part of the cycle is something you can influence.

This is not the same as saying the pain is your fault. You did not choose this, and you did nothing wrong. But just as someone with diabetes or high blood pressure gets the best results by taking an active part in their care, people with pelvic pain who take an active role do the best. Medications and procedures can help, but on their own they rarely solve this problem. **The most powerful treatments are the ones you practise yourself, every day.**

The recovery plan at a glance

Most successful recovery plans include the following building blocks. The chapters that follow explain each one.

1. **Understand your pain.** You are already doing this.
2. **Release the pelvic floor.** Usually with the help of a pelvic floor physiotherapist (Chapter 8).
3. **Practise relaxation every day.** Breathing, letting go of the pelvic floor, and calming the nervous system (Chapter 9).
4. **Calm the alarm system.** Sleep, mood, worry and support (Chapter 10).
5. **Change the daily habits that feed tension.** Sitting, bladder and bowel habits, exercise and sex (Chapter 11).
6. **Use medications and other treatments wisely**, as helpers rather than cures (Chapter 12).
7. **Have a plan for flare-ups**, so they don't throw you off course (Chapter 13).

What recovery looks like

Recovery from chronic pelvic pain is rarely a straight line. More often it looks like a jagged line that trends upward: good weeks and bad weeks, with the bad weeks gradually becoming shorter, milder and further apart. Many people notice that they are *doing* more before they notice they are *hurting* less.

Most people begin to see meaningful change within **two to three months** of consistent practice and physiotherapy, with continued improvement over **six to twelve months**. Some people recover completely. Others reach a point where symptoms are mild and occasional, and no longer run their lives. Both are real recoveries.

It helps to measure progress over weeks rather than days. A useful question is not “Does it hurt today?” but “Compared with a month ago, am I having more good days? Am I doing more of what matters to me? Are my flares shorter?” There is a simple weekly tracker at the end of this guide.

Key points

- The pain is not your fault, but you can strongly influence your recovery.
- Daily self-management, combined with physiotherapy, is the core of treatment.
- Recovery is uneven. Measure it over weeks and months, not days.
- Most people see real change within two to three months, and keep improving for a year or more.

CHAPTER 8

Pelvic floor physiotherapy

For most people with chronic pelvic pain, a **pelvic floor physiotherapist** is the most important member of the care team. These are physiotherapists with extra training in the muscles of the pelvis. In research studies, hands-on pelvic floor physiotherapy has helped substantially more people than general massage, and current urology guidelines recommend it for people with tender pelvic floor muscles.

What happens at physiotherapy

Your first visit is usually an hour long. The physiotherapist will ask about your symptoms, bladder, bowel and sexual function, and daily routine. They will look at your posture, breathing, hips and back, which often contribute to pelvic tension.

With your permission, they will usually examine the pelvic floor muscles directly. In men this is done with a gloved finger through the rectum; in women, usually through the vagina, and sometimes the rectum. This lets the physiotherapist find tight bands and trigger points, see how well you can tighten and relax, and often reproduce your familiar pain, which can be a relief in itself, because it confirms where the pain is coming from.

The idea of an internal exam can feel embarrassing or frightening. That is completely understandable. Pelvic floor physiotherapists do this every day, they will explain each step, and you are always in control: you can ask them to slow down, stop, or leave the internal exam for another visit. If you have had difficult experiences in the past, it is fine to tell them so. You can also bring a support person.

Treatment usually includes

- **Manual therapy:** gentle, sustained pressure and stretching to release tight muscles and trigger points, both inside and outside the pelvis (belly, inner thighs, buttocks, hips and low back).
- **Down-training:** learning to feel the difference between tight and relaxed, and to let the pelvic floor drop and lengthen on purpose. Sometimes this uses biofeedback, where a small sensor shows your muscle activity on a screen.
- **Breathing retraining:** because the diaphragm and pelvic floor move together.
- **Stretching and movement:** to loosen the hips, inner thighs and low back.
- **A home program:** exercises and, in some cases, self-release with your own hand or a specially designed wand, or graduated vaginal trainers.
- **Advice on daily habits:** sitting, toileting, exercise and sex.

How long it takes

Most people need a series of visits, often every one to two weeks at first, and then less often. Many notice some improvement within the first four to six visits. The home program between visits matters as much as the visits themselves.

It is common to feel sore for a day or two after hands-on treatment, much like after a deep massage. That is not a sign of harm. Tell your physiotherapist if soreness lasts longer, so they can adjust the treatment.

Finding a pelvic floor physiotherapist

In Ontario you do not need a doctor's referral to see a physiotherapist, although some insurance plans require one for coverage. Pelvic floor physiotherapy at private clinics is usually not covered by OHIP; many extended health plans cover part of the cost. If you are 65 or older, you may be able to get OHIP-funded physiotherapy at some community clinics, although not all of them offer pelvic floor care. When you call a clinic, ask specifically whether they treat **pelvic pain** and **pelvic floor tension** (and, for men, whether they treat **male pelvic pain**), since some clinics focus mainly on pregnancy and childbirth. Our office keeps a list of clinics in London and southwestern Ontario on the Male pelvic pain page (nicholaspowerurology.com/education/male-pelvic-pain) of this website.

If physiotherapy is not available to you, or not affordable, the daily practices in the next chapters are still very worthwhile on their own.

CHAPTER 9

Learning to let go: your daily practice

If there is one habit that will change your pelvic pain, it is this: **a daily practice of deliberately relaxing your body, your breath and your pelvic floor.**

A tight pelvic floor will not loosen just because you know it is tight. These muscles have spent months or years learning to hold on, and the nervous system has learned to keep them that way. Retraining them takes regular, repeated practice, just as learning a language or an instrument does. A few minutes now and then is not enough. Daily practice, kept up over months, is what changes the pattern.

Aim for:

- **one longer session of 20 to 30 minutes every day** (two sessions is even better when you are in a flare), lying down somewhere quiet; and
- **several short "check-ins" throughout the day**, of 30 seconds to two minutes, wherever you are.

Below are the building blocks. Start with the first one and add the others over a week or two. Many people find it easiest to follow a recording. You can make your own by reading these instructions aloud slowly into your phone, or use one of the apps listed in the resources section.

PRACTICE 1

Belly breathing

Your diaphragm, the dome-shaped breathing muscle under your ribs, and your pelvic floor move together like two ends of a piston. When you breathe in fully and slowly, the diaphragm moves down and the pelvic floor gently drops and widens. When you breathe out, both lift back up. People who hold tension tend to breathe high in the chest, which keeps the pelvic floor braced. Slow belly breathing is the quickest way to start letting it go.

1. Lie on your back with your knees bent and feet flat, or with a pillow under your knees. Rest one hand on your chest and one on your belly.
2. Breathe in slowly through your nose for a count of about four. Let your belly and the sides of your lower ribs rise and widen under your hand. Your chest should barely move.
3. Breathe out slowly through your mouth, as if fogging a mirror or sighing, for a count of about six. Let your belly fall.
4. Keep going for five minutes. If your mind wanders, which it will, gently come back to counting the breath.

The long, slow breath out is the part that calms the nervous system most. Don't force big breaths; slow and easy is better than deep and effortful.

PRACTICE 2

Letting the pelvic floor drop

Once your breathing has settled, bring your attention to the pelvic floor.

1. As you breathe in, imagine the area between your sitting bones softening and widening, like a balloon filling gently from below, or a flower opening.
2. Picture the feeling of just starting to pass urine or gas, and let the muscles soften in that direction. Don't push or strain; simply *let go*.
3. As you breathe out, let the muscles stay soft. Don't lift or squeeze.
4. Continue for five to ten breaths. Then rest and just breathe.

Many people find they can't feel much at first, or that they can feel the muscles but they won't release. That is normal and expected. You are learning a skill that has been out of use. Some people find it helpful to sit on a rolled towel, or to put a warm pack over the perineum, to bring awareness to the area. A physiotherapist can help you find the right feeling.

PRACTICE 3

The body scan: noticing without fighting

This practice teaches you to release tension throughout the body, and to change your relationship with discomfort.

1. Lying comfortably, breathing slowly, bring your attention to your forehead and eyes. Notice any tension there. Then, on a breath out, let it soften.
2. Move slowly down the body: jaw (let the teeth part and the tongue rest), throat, shoulders, arms and hands, chest, belly, low back, buttocks, pelvic floor, thighs, calves and feet. Spend a few breaths in each area.
3. In each place, simply notice what is there. If you find tension, invite it to soften on the out-breath. If it doesn't soften, that is fine too. Notice it, and move on.
4. When you reach the pelvis, you may notice discomfort. Try not to brace against it or push it away. Instead, let your attention rest gently on it, as if you were watching weather pass, and keep breathing slowly. Allow the sensation to be there.

That last step can feel strange. Our instinct with pain is to fight it, tense against it, or run from it. But bracing against pain is exactly what tightens the pelvis further. Learning to stay soft and calm in the presence of discomfort, rather than tensing against it, is one of the most powerful skills in pelvic pain recovery. It takes practice. Over time, many people find that discomfort they can observe calmly becomes smaller and less frightening.

This approach is related to methods called **paradoxical relaxation** and **mindfulness**, which have been studied in chronic pelvic pain. If you want to go deeper, the resources section lists some places to start.

PRACTICE 4

Tension check-ins during the day

Most pelvic tension happens when you aren't paying attention: at your desk, in traffic, in a tense conversation, while scrolling on your phone. Short check-ins build awareness and interrupt the habit.

1. Several times a day, pause and ask: *Where am I holding on right now?*
2. Check your **jaw** (let the teeth part), **shoulders** (let them drop), **belly** (let it soften), **buttocks** (unclench), and **pelvic floor** (let it drop).
3. Take three slow belly breaths, with long breaths out.

Link check-ins to things you already do: each time you sit down at your desk, stop at a red light, wash your hands, finish a phone call or open your email. Some people use a phone reminder every hour, or a small sticker on their watch or computer as a cue.

PRACTICE 5

Gentle stretches

Tight hips, inner thighs and buttocks pull on the pelvic floor. These gentle stretches can be done once or twice a day, ideally after warming up with a short walk or a warm shower. Move slowly, breathe out into each stretch, and stay within a range that feels like a comfortable pull, not pain. Hold each for 30 to 60 seconds while belly breathing.

- **Child's pose:** kneel, sit back toward your heels with knees apart, and rest your forehead and arms forward on the floor or a pillow. Breathe into your back and pelvis.
- **Happy baby:** lying on your back, bring your knees toward your armpits and hold the backs of your thighs or your feet, letting the knees widen. Rock gently side to side if that feels good.
- **Reclined butterfly:** lying on your back, put the soles of your feet together and let your knees fall out to the sides, with pillows under the knees for support.
- **Figure-four stretch:** lying on your back with knees bent, cross one ankle over the opposite knee, then gently draw the lower knee toward your chest until you feel a stretch in the buttock. Switch sides.
- **Supported deep squat:** holding onto a counter or doorframe, lower into a comfortable squat with feet wide and heels down (or on a rolled towel), and let your pelvis relax as you breathe. Come up slowly.

Avoid forcing stretches, bouncing, or stretching through sharp pain. If any stretch increases your pelvic pain, skip it and ask your physiotherapist for an alternative.

Making it stick

- **Same time, same place.** Many people practise before bed, or as soon as they get home from work.
- **Expect it to feel like nothing is happening** for the first few weeks. That is normal. You are laying down a new habit in the nervous system, and the benefits build up over time.
- **Don't judge each session.** Some sessions will feel calm and others restless. Both count.
- **Keep going on good days.** Practice on good days is what makes the bad days less frequent.

CHAPTER 10

Calming the alarm system

Because the nervous system is so central to chronic pelvic pain, looking after it is part of treatment, not an extra.

Sleep

Poor sleep turns up pain sensitivity, and pain disturbs sleep: another cycle. Protecting sleep is one of the most effective things you can do.

- Keep regular sleep and wake times, including on weekends.
- Make the bedroom dark, cool and quiet, and use it for sleep and intimacy only.
- Put screens away 30 to 60 minutes before bed; this is a good time for your relaxation practice.
- Limit caffeine after noon and alcohol in the evening, both of which disturb sleep and can irritate the bladder.
- If you wake at night and can't sleep, get up and do something calm in dim light until you feel sleepy.
- If insomnia persists, ask your doctor about cognitive behavioural therapy for insomnia (CBT-I), which works better than sleeping pills over the long term.

Worry and catastrophic thinking

It is completely understandable to worry when you have pain in such a private, important part of the body. But some patterns of thought make pain worse, and they are worth noticing:

- **Imagining the worst:** "This must be cancer." "I'll never have a normal sex life." "It's getting worse; it will always be like this."
- **Constant scanning:** checking the pelvis for sensations throughout the day.
- **Searching:** hours on forums and websites, which tend to collect the worst stories.

When you notice one of these, try to name it ("That's my worried brain again"), take a few slow breaths, and remind yourself of what you know: *My tests have been reassuring. This is muscle tension and a sensitive alarm system. Pain is not damage. Flares pass.* Then turn your attention to something else that matters to you.

Consider limiting internet searching about your symptoms, or stopping altogether for a while. Many patients tell me this alone made a noticeable difference.

Counselling and pain psychology

Seeing a psychologist or counsellor is not a sign that your pain is imaginary or that you are weak. Pain psychology is a standard part of treatment for chronic pain of all kinds, and current guidelines include it for chronic pelvic pain. Approaches that help include:

- **Cognitive behavioural therapy (CBT):** practical skills for working with unhelpful thoughts, worry and avoidance;
- **mindfulness-based programs:** structured training in calm, non-judging attention, often over eight weeks;
- **acceptance and commitment therapy (ACT):** building a full life around your values, even while some symptoms remain.

Counselling is especially worth considering if you have significant anxiety, low mood, past trauma, or if pain is affecting your relationships. If you ever feel hopeless or have thoughts of harming yourself, please tell your doctor right away, call or text 9-8-8 (the Suicide Crisis Helpline, available 24 hours a day across Canada), or go to an emergency department.

Movement and enjoyment

Regular gentle aerobic exercise, such as walking, swimming or an elliptical machine, is one of the best natural ways to calm a sensitized nervous system. Start with what is comfortable, even ten minutes, and build gradually. Spend time on things you enjoy and that absorb your attention: hobbies, music, time outdoors, time with people you like. These are not distractions from treatment. They are treatment.

CHAPTER 11

Everyday habits that make a difference

Sitting

- Get up and move every 30 to 45 minutes. Set a reminder if you need to.
- Try a cushion with a cut-out at the back (sometimes sold as a “coccyx cushion”) or a U-shaped cushion that takes pressure off the perineum. Some people prefer to sit on two folded towels placed under each thigh, leaving the middle free.
- Sit evenly on both sitting bones with your feet flat. Avoid perching on the edge of a chair, crossing your legs for long periods, or slumping onto your tailbone.
- Consider a sit–stand desk.
- For long drives, stop every hour to walk and stretch.

Bladder habits

- **Drink enough water**, spread through the day, so your urine is pale yellow. Restricting fluids makes urine concentrated and more irritating to the bladder.
- **Identify your bladder irritants.** Common ones are coffee, tea, cola and energy drinks, alcohol, carbonated drinks, artificial sweeteners, citrus juices, tomato and spicy foods. Not everyone reacts to all of them. Try cutting out the likely culprits for two to three weeks, then bring them back one at a time to see which matter for you.
- **Avoid “just in case” trips** to the bathroom when you don’t need to go. Frequent emptying trains the bladder to hold less.
- **Don’t push or strain to pee.** Sit down (men too, if it helps), relax your belly and pelvic floor, breathe out slowly, and let the urine come on its own. Give yourself time.
- **If urgency strikes**, stand or sit still, breathe slowly with long breaths out, and let the urge peak and fade before walking calmly to the bathroom. If your bladder symptoms are prominent, the bladder retraining section of our pelvic floor and bladder handouts (nicholaspowerurology.com/education/kegels) explains how to stretch the time between trips gradually. If you have pelvic pain, use the calm, slow breathing described above when an urge hits rather than the quick squeezes suggested in the handouts, and skip the strengthening exercises unless your physiotherapist recommends them.

Bowel habits

Constipation and straining are among the most common and most overlooked drivers of pelvic pain. Hard stools and pushing force the pelvic floor to work against itself.

- Aim for soft, easy-to-pass bowel movements. Increase fibre gradually (vegetables, fruit, whole grains, beans, ground flaxseed), and drink enough water.
- If diet alone isn't enough, ask your doctor or pharmacist about a fibre supplement (taken with a full glass of water) or a gentle daily laxative such as polyethylene glycol.
- Put your feet on a small footstool so your knees are higher than your hips. Lean forward with your elbows on your knees, let your belly relax and bulge, and breathe out as you go.
- Don't strain, hold your breath or sit on the toilet for long periods (leave your phone outside).
- Go when you feel the urge, rather than putting it off.

Exercise

- **Usually helpful:** walking, swimming, elliptical machines, gentle yoga, Pilates taught by an instructor who understands pelvic pain, and stretching.
- **Often aggravating, at least for a while:** cycling (or use a cut-out seat and stand often), heavy lifting with breath-holding, high-intensity "core" workouts, lots of sit-ups or crunches, and rowing machines.
- When you lift anything heavy, **breathe out during the effort** instead of holding your breath.
- Return to more demanding activities gradually once your symptoms have settled, and expect some minor ups and downs as you do.

Sex and intimacy

Pelvic pain often affects sex, and worry about sex often feeds pelvic pain. A few principles help:

- **Sex and ejaculation do not damage anything.** Pain after ejaculation or orgasm is common in pelvic pain. It comes from the pelvic floor muscles contracting strongly and then not releasing well afterward. It is not harmful and it usually improves as the muscles recover.
- Try a warm bath or your relaxation practice afterward to help the muscles let go.
- Take the pressure off. It is fine to take intercourse off the table for a time and focus on touch and closeness that feels good, then return gradually.
- For women, use plenty of good-quality lubricant, and take time for arousal, which naturally relaxes the pelvic floor. If penetration is painful, a physiotherapist can guide you through a graduated program.
- Talk openly with your partner. Pain is easier to manage when your partner understands it and you are working on it together. Sharing this guide can help.
- Changes in erections or arousal are common with pelvic pain and stress, and usually improve as the pain improves. Your doctor can help if they don't.

Warmth and comfort

- Warm baths, or a warm pack over the perineum or lower belly, often ease muscle tension. Many people find a bath before their relaxation practice especially helpful.
- Wear loose, comfortable underwear and pants. Avoid tight jeans, tight waistbands and tight cycling shorts.

CHAPTER 12

Medications and other treatments

Medications and procedures can be useful **helpers** in recovery. They can turn down the volume enough to let you sleep, exercise and practise relaxation, and they can relieve specific symptoms. But for most people they are not a cure on their own. The best results come from combining them with the self-management and physiotherapy described earlier.

Because chronic pelvic pain is different from person to person, treatment is tailored to your main symptoms. Not every treatment below is right for everyone, and your doctor will discuss the benefits and side effects of anything they recommend. It is normal to try a treatment for a set period (often 6 to 12 weeks), then keep what helps and stop what doesn't.

For urinary symptoms

- **Alpha-blockers** (such as tamsulosin, alfuzosin or silodosin) relax the muscle at the bladder outlet and prostate. They can help men with a weak stream, hesitancy or frequent urination, especially when symptoms are relatively recent. Common side effects are dizziness when standing up and a change in ejaculation.
- **Medications for bladder pain syndrome**, such as amitriptyline, hydroxyzine, cimetidine or pentosan polysulfate, may be offered when the bladder is the main source of pain. Pentosan polysulfate can damage the retina (the back of the eye), especially with long-term use, so you need an eye exam before starting it and regular eye exams while taking it.
- **Bladder instillations**, where medicine is placed directly into the bladder through a small catheter, may be used for bladder pain that does not respond to other treatments.

For pain and nerve sensitivity

- **Anti-inflammatories** (such as ibuprofen or naproxen) can help during flares. They are best used for short periods because of their effects on the stomach, kidneys and heart. Check with your doctor or pharmacist that they are safe for you.
- **Nerve-calming medications** (such as low-dose amitriptyline or nortriptyline, duloxetine, gabapentin or pregabalin) can reduce the sensitivity of the alarm system and often improve sleep. They are started at a low dose and increased slowly, and they take several weeks to work. They are used for their effect on nerves, not because your pain is "psychological."
- **Opioid pain medications** are generally not recommended for chronic pelvic pain. Over time they can make pain sensitivity worse and cause constipation, which aggravates pelvic pain.

Supplements

- **Quercetin** (a plant flavonoid) and **rye pollen extract** have some evidence of benefit in men with chronic prostatitis/pelvic pain, and are generally well tolerated. Ask your doctor or pharmacist before starting them, especially if you take other medications.

Other treatments

- **Acupuncture** has reasonable evidence in chronic prostatitis/pelvic pain and is a reasonable option for many people.
- **Low-intensity shockwave therapy**, applied to the perineum, has shown benefit in men with chronic pelvic pain in several trials. It is not covered by OHIP.
- **Trigger point or nerve block injections**, and in some cases **Botox** injections into the pelvic floor muscles, may be used for pain that has not responded to other treatments, often to give a window of relief that makes physiotherapy more effective.
- **Referral to a chronic pain program** may be suggested if pain remains severe despite these steps.

What generally does not help

- **Repeated courses of antibiotics** when urine and other cultures are negative.
- **Surgery on the prostate or bladder** to relieve pelvic pain, which does not address the muscles and nerves and can make things worse. (The exception is a specific finding such as Hunner lesions, a type of sore seen inside the bladder in some people with bladder pain, which is treated with a scope procedure.)
- **Long-term steroid tablets.**
- **Pelvic floor strengthening (Kegel) exercises**, unless a physiotherapist has assessed you and specifically recommends them.
- **Searching for a single "magic bullet."** The people who do best combine several modest helps, practised consistently.

CHAPTER 13

Handling flare-ups

Flares are part of recovery for almost everyone. A flare does not mean you are back to square one, that your treatment has failed, or that something new is wrong. It means the alarm system and the muscles have been stirred up, and they will settle again.

Common triggers include a stressful week, poor sleep, long periods of sitting or travel, constipation, illness, sex after a long break, a new or intense workout, alcohol or bladder-irritating drinks, and sometimes nothing obvious at all. Trying to find a cause for every flare can itself become a source of worry. Sometimes flares just happen.

A flare plan

Having a plan ready before a flare starts helps you respond calmly instead of with alarm. Use the worksheet at the end of this guide to write your own. A good plan often includes:

1. **Remind yourself:** *This is a flare. It is not damage. It will pass. I have been here before and come out the other side.*
2. **Double your relaxation practice:** two longer sessions a day, and frequent check-ins.
3. **Use warmth:** a warm bath or a warm pack, once or twice a day.
4. **Keep moving gently:** short walks rather than bed rest or hard workouts.
5. **Reduce sitting** and use your cushion.
6. **Look after your bladder and bowels:** drink water steadily, avoid your bladder irritants, and keep stools soft.
7. **Use the medications your doctor has suggested for flares**, if any.
8. **Avoid the traps:** internet searching, repeated checking, cancelling everything, or starting new treatments in a panic.
9. **Be kind to yourself.** Notice self-critical thoughts (“I’ve ruined everything”) and answer them as you would answer a friend.

Most flares settle within a few days to a couple of weeks. As recovery continues, they usually become milder, shorter and less frequent. If a flare is very different from your usual pattern, or comes with any of the warning signs in the next section, contact your doctor.

CHAPTER 14

When to get help sooner

Chronic pelvic pain is not dangerous, but it doesn't protect you from other problems. Don't assume that every new symptom is part of your pelvic pain. Contact your doctor promptly, or go to an emergency department, if you have:

- **fever** (38°C or higher) or shaking chills with pelvic pain, or burning with urination;
- **inability to pass urine**, or passing only small amounts with a full, painful bladder (go to an emergency department);
- **blood in your urine** that you can see;
- **blood in your semen** that is new or keeps happening;
- a **new lump** or swelling in a testicle, the scrotum or the groin, or sudden severe testicular pain (go to an emergency department);
- **bleeding from the rectum**, or a lasting change in bowel habits;
- for women, **bleeding after menopause**, or bleeding after sex;
- **numbness** in the groin, buttocks or genitals, new **leg weakness**, or new loss of control of your bladder or bowels (go to an emergency department);
- **unexplained weight loss** or night sweats;
- pain that is **sudden, severe**, or clearly different from your usual pattern.

CHAPTER 15

Working with your care team, and further resources

Chronic pelvic pain is best treated by a team. Depending on your needs, yours may include your family doctor, your urologist, a pelvic floor physiotherapist, a counsellor or pain psychologist, and sometimes a gynaecologist, gastroenterologist or chronic pain program. You are the most important member of that team.

Some tips for appointments:

- Bring your weekly tracker so you can describe how things are changing over time.
- Write down your two or three most important questions in advance.
- Tell us what matters most to you: sleep, work, sex, exercise, travel. Your goals shape your plan.
- Let us know which treatments have helped and which haven't, so we can stop what isn't working.

Further reading and resources

- ***A Headache in the Pelvis***, by David Wise and Rodney Anderson. A well-known book on pelvic floor tension and paradoxical relaxation, written for patients. Many of my patients have found it very helpful.
- **Pelvic Health Solutions** (pelvichealthsolutions.ca). A Canadian organization that trains pelvic health physiotherapists; its website includes a directory to help find a pelvic health physiotherapist near you.
- **Interstitial Cystitis Association** (ichelp.org). Information and support for bladder pain syndrome.
- **Prostatitis Foundation** (prostatitis.org). Information and support for men with chronic prostatitis/pelvic pain.
- **Mindfulness and relaxation apps:** Insight Timer (free) has many guided body scans and relaxation recordings. Search for "body scan" or "progressive relaxation."
- **Pain education apps:** Curable is an app that teaches pain science and self-management skills for chronic pain.
- **Our website:** the Male pelvic pain (nicholaspowerurology.com/education/male-pelvic-pain) page has more detail on diagnosis and treatment and a list of pelvic floor physiotherapy clinics in southwestern Ontario. The Kegel exercises (nicholaspowerurology.com/education/kegels) page covers bladder retraining, with printable handouts.

A FINAL WORD

Recovery is possible

If you take away only a few things from this guide, let them be these:

- Your pain is real, it is common, and it has an explanation.
- It is driven mostly by tight pelvic floor muscles and a sensitive nervous system, not by infection, cancer or damage.
- Hurt does not mean harm. Flares pass.
- The most powerful treatments are the ones you practise every day: letting go of tension, calming the nervous system, and gradually returning to the life you want.
- Recovery takes months, it is rarely a straight line, and it is very possible.

I have watched many patients who were frightened, frustrated and worn down by pelvic pain gradually take their lives back. It almost always starts the same way: with understanding, and with a decision to start practising. You have already taken the first step by reading this.

WORKSHEETS

Your recovery tools

My daily practice

Tick each day you do it. Aim for most days, not perfect days.

Practice	Mon	Tue	Wed	Thu	Fri	Sat	Sun
Long relaxation session (20–30 min)							
Tension check-ins (how many?)							
Gentle stretches							
Walk or other gentle exercise							
Got up from sitting every 30–45 min							
Soft bowel movement, no straining							

My weekly tracker

Fill this in once a week, on the same day. Look for the trend over a month or more, not the change from one week to the next.

Week of	Average pain (0–10)	Good days this week (0–7)	Flares (how many, how long)	Something I did that matters to me

My flare plan

Fill this in when you are feeling well, and keep it somewhere easy to find.

My early warning signs of a flare:

What I will remind myself:

This is a flare, not damage. It will pass. I have handled this before.

My relaxation plan during a flare (when, how long):

Comfort measures that help me (heat, bath, cushion, walking):

Medications my doctor has suggested for flares:

Things I will avoid (searching, checking, cancelling everything):

People I can talk to:

I will contact my doctor if:

I have any of the warning signs in Chapter 14, or this flare is very different from my usual pattern.

My goals

What would you like to get back to, or do more of, as your pain improves? Choose things you can see and measure, such as “walk for 30 minutes,” “sit through a movie,” or “enjoy intimacy with my partner.”

In the next month:

In three months:

In six months:
